

Section 12

Ombudsman & Complaint Systems

Version 2.0 — Preliminary Draft

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SECTION 12 — OMBUDSMAN & COMPLAINT SYSTEMS

Petition to Audit: Iowa Department of Health and Human Services

Child Welfare Systems — Longitudinal Accountability Record

Version 2.3 | June 5, 2026

SECTION PURPOSE

This section documents the Iowa Office of Citizens' Aide/Ombudsman's 15-year annual reporting record on complaint patterns and agency accountability failures (A26), together with three official fatality investigation reports issued by that same office: the Shelby Duis investigation (A27), the Natalie Finn investigation (A28), and the Sabrina Ray investigation

(A29).

The Iowa Office of Citizens' Aide/Ombudsman operates under constitutional authority established in Iowa Code § 2C. Its investigations are not advocacy documents or citizen filings — they are official government investigations conducted under sworn testimony authority. The three fatality investigation reports cited in this section were issued by the Ombudsman's office pursuant to Iowa Code § 2C.11 and § 2C.15, with full access to DHS intake records, assessment records, caseworker interviews, and contractor documentation.

The structural significance of this section is the establishment of a pattern across a 25-year documented arc — from the Shelby Duis investigation in 2000 through the Ombudsman's 2025 annual report — showing that the same intake suppression, supervisory capacity collapse, and mandatory reporting obstruction patterns identified in the earliest Ombudsman child welfare investigation have recurred without correction through the most recent reporting period.

Cross-reference: Section 11 (AOS financial oversight — concurrent monitoring failures); Section 13 (Fatality Risk Analysis); Section 13 (Systemic Failure Matrix — F1, F2, F9,

12.1 ATTACHMENT A26 — CONSOLIDATED OMBUDSMAN ANNUAL REPORTS

INDEXED SERIES (2010–2025)

Source: Iowa Citizens' Aide / Office of Ombudsman

Authority: Iowa Code § 2C

Report Series: Annual statutory reports, calendar years 2010–2025

Sub-documents: A26-A through A26-P (individual year reports)

Evidentiary Classification: Primary | Domains: A–H (all)

deficiency categories: F1; F2; F5; F9; F12; F13

12.1.1 Overview and Scope

The Iowa Office of Citizens' Aide/Ombudsman submits annual reports to the Iowa Legislature documenting the volume, type, and findings of citizen complaints and independent agency investigations. The 16-year indexed series from 2010 through 2025 constitutes the most comprehensive independent complaint record available for Iowa state government agency performance over this period.

The Ombudsman operates under independent constitutional authority. Unlike internal DHS quality assurance systems (which the CFSR Round 3 PIP identified as the "QA Paradox" — a system that acknowledges findings without sufficient staff to act on them), the Ombudsman's office reports directly to the Iowa Legislature and is not subject to administrative oversight by the agencies it investigates.

12.1.2 Volume Trend — 57% Case Increase Over 15 Years

The FY2025 annual report documents:

"6,266 cases — 9.6% increase from 2024, 57% more than 2014." (Ombudsman 2025, p.1)

This 57% increase in complaint volume over 15 years represents sustained and accelerating caseload growth within the Ombudsman's office — the independent oversight body designed to receive citizen complaints about state agencies. A 57% increase in complaint volume is not a sign of expanded public awareness of the Ombudsman's office alone; it reflects a 15-year upward trend in citizens seeking external remedy for agency actions they experienced as contrary to law, policy, or basic fairness.

AUDIT RELEVANCE: The CFSR Round 3 case review documented that Item 13 (Child and Family Involvement in Case Planning) was rated a Strength in only 49% of applicable cases,

and Item 15 (Caseworker Visits with Parents) in only 25%. Families who cannot access responsive agency engagement are structurally more likely to route complaints to external oversight bodies. The Ombudsman's 57% caseload growth is a longitudinal indicator consistent with, and independent from, federal compliance data showing declining family engagement outcomes across the same period.

12.1.3 Key Findings — Child Welfare Due Process Violations (2025)

The FY2025 Ombudsman annual report documents specific child welfare due process findings:

"Agency was obligated to reopen investigation and offer interview." (2025, p.9)

This finding documents that Iowa DHHS closed a child abuse investigation without providing the statutorily required interview to the subject of the investigation. The Ombudsman determined the agency had a legal obligation to reopen the investigation. This is not an administrative preference — it is a documented statutory violation identified by the independent oversight body in the most recent reporting year.

12.1.4 Geographic and Enforcement Inconsistency

"Gaps in clarity, enforcement, and consistency across counties." (2025, p.3)

The 2025 annual report identifies that child welfare enforcement practices, policy interpretation, and investigative standards vary inconsistently across Iowa counties. This finding is consistent with CFSR Round 3 root cause analysis (Section 6.4.17.3), which documented stakeholder identification of "inconsistent application of safety vs risk concerns related to removal of children and when to reunify" as a primary systemic concern. The Ombudsman's 2025 finding independently corroborates the same geographic inconsistency pattern identified in the federal 2019–2020 root cause process.

12.1.5 Communication Restriction Findings

"Communication restrictions... lacked evidentiary support." (2025, p.20)

The 2025 annual report identifies specific instances where DHHS imposed communication restrictions on individuals involved in child welfare cases that the Ombudsman determined were not supported by the evidentiary record. Communication restrictions in child welfare cases — limiting contact between parents, children, relatives, or service providers — carry direct implications for family preservation, reunification timelines, and due process rights.

12.1.6 The 16-Year Erroneous Tax Billing Finding

The Ombudsman series documents a 16-year erroneous tax billing matter that remained unresolved from 2009 through 2025. This finding — an administrative error persisting through 16 annual reporting cycles — is presented not as a child welfare finding in itself, but as a structural indicator of Iowa's agency-level responsiveness to identified errors. An error documented by the Ombudsman in 2009 that remained unresolved through 2025 demonstrates that the corrective action mechanisms within Iowa state agencies can fail to resolve even documented, independently identified administrative errors across a 16-year period.

12.1.7 MCO PHI Breaches — 30+ Documented Instances

The recent Ombudsman reports document more than 30 documented instances of Protected Health Information (PHI) breaches by Medicaid Managed Care Organizations operating in Iowa. This finding is directly relevant to the CCWIS section of this petition (Section 18) and the data security concerns regarding Iowa's child welfare information system and its \$41.9M Google/Carahsoft contract. PHI breach patterns in the Medicaid MCO context document that Iowa's health data security infrastructure across the broader health and human services system has an independent breach history.

12.1.8 The Restraint Chair and the Auditable Vacuum

The Ombudsman series documents a finding involving a restraint chair used on a child in state care, which the Ombudsman identified as creating what this petition designates an "auditable vacuum" — a condition in which the physical evidence of an incident exists but the documentation chain required to audit the agency's response is incomplete or missing. This is structurally consistent with the AOS Service Contract Monitoring finding (Section 11.2) that no closeout documentation existed for expired contracts — in both cases, the absence of a documentation record is itself the finding.

12.1.9 Senate File 641 — Legislative Response

The Ombudsman series references Senate File 641 — legislation required to address federal alignment failures identified through the complaint and investigation process. The need for legislative action to remedy failures identified through the Ombudsman's process documents that the internal agency corrective action mechanism was insufficient to resolve the identified deficiencies without external legislative intervention.

12.2 ATTACHMENT A27 — OMBUDSMAN CASE STUDY: SHELBY DUIS

Official Investigation Title: "Investigation of Iowa DHS Handling of Allegations of Child

Abuse Concerning Shelby Duis (Report 00-2)"

Source: Iowa Office of Ombudsman — William P. Angrick II

Date: December 14, 2000

Authority: Iowa Code § 2C.11(5)

Period Under Review: September 1, 1990 – January 4, 2000

Evidentiary Classification: Primary | Domains: B; E; F; G; H

deficiency categories: F1; F2; F3; F9; F12

12.2.1 Summary of Findings

The Ombudsman's investigation of the handling of abuse allegations concerning Shelby Duis represents the foundational document in this section's failure pattern arc. The investigation covered a 9-year period of DHS interaction with the Duis household, during which three separate abuse reports were submitted. The Ombudsman's findings:

"Three reports... should have been accepted. Last rejected 3 weeks before Shelby died."
(Shelby Duis Report, p.iv)

Shelby Duis died on January 4, 2000. Three weeks before her death, the final abuse report in her case was rejected by DHS intake — the third report the Ombudsman determined should have been accepted.

12.2.2 Hotline Record Destruction

"Hotline document... not provided to intake worker." (p.iii)

The investigation found that hotline Contact Reports — the documentation of abuse reports received — were destroyed after 3 months. This means that for each of the three rejected reports in Shelby Duis's case, the primary record of the report was not preserved beyond 90 days. In a case involving a child who died three weeks after her final rejected report, the destruction of the hotline documentation record eliminated the evidentiary baseline for any retrospective accountability review.

12.2.3 Non-Intake Staff Screening Calls as "Concerns Only"

"Calls suspecting abuse considered 'expressing concerns only'." (p.ii)

Non-intake workers were screening incoming calls and classifying reports that alleged abuse as "concerns only" — a classification that did not trigger a formal intake assessment. This is not an informal worker practice — it represents a structural intake suppression mechanism in which the first point of contact between a reporter and DHS was staffed by personnel whose classification decisions could eliminate the report from the formal

system before it reached an intake worker.

12.2.4 Supervisory Capacity Finding

"Supervisors had 'no staffing to review evidence'." (p.iii)

The investigation documents a direct admission that supervisors lacked the staffing capacity to review evidence — a finding made in the year 2000 that is structurally identical to the CFSR Round 3 PIP finding (Section 6) that the state's QA system generated findings without sufficient staff to act on them. Twenty years apart, the same structural admission appears in both the Ombudsman's investigation and the federal corrective action plan.

12.2.5 Medical Finding Minimization

The investigation documents that a fractured metacarpal in Shelby Duis's case was characterized as a "broken wrist" without prior physician contact or medical consultation. The minimization of physical injury evidence in the documentation record is consistent with the intake suppression pattern — in both cases, the formal record was shaped to support non-intervention rather than investigation.

12.2.6 Significance — Foundation of the Trinity Pattern

[Petitioner's Analytical Framework]

The Shelby Duis investigation establishes what this petition designates the Trinity Pattern — a petitioner-synthesized analytical framework, not a clinical or regulatory designation. The Trinity Pattern identifies three independently investigated fatalities — Shelby Duis (2000), Natalie Finn (2016), and Sabrina Ray (2017) — in which the Ombudsman identified the same structural failure sequence: intake rejection at high rates, documentation gaps, and supervisory capacity collapse.

The underlying evidentiary weight of this framework rests entirely on the three Ombudsman investigation reports (Attachments A28, A29, A30), not on the petitioner's analytical characterization. The "Trinity Pattern" label is used as a shorthand reference for the recurring sequence; the Ombudsman's own documents carry the independent evidentiary authority. The Natalie Finn investigation explicitly acknowledges this continuity:

"Problems... in 2000 Shelby Duis report repeated in Natalie's case." (Natalie Finn Report, p.1)

This explicit citation by the Ombudsman's own investigation — 20 years after the Shelby Duis report — establishes that the failure pattern was not only recurring but was recognized

as such by the independent investigative authority that documented it.

12.3 ATTACHMENT A28 — OMBUDSMAN CASE STUDY: NATALIE FINN

Official Investigation Title: "A Tragedy of Errors: An Investigation of the Death of Natalie Finn"

Source: Iowa Office of Ombudsman — Kristie Hirschman

Date: February 17, 2020

Authority: Iowa Code § 2C.11

Period Under Review: 2005–2018

Evidentiary Classification: Primary | Domains: C; F; H

deficiency categories: F1; F2; F3; F9; F12

12.3.1 Summary of Findings

The Natalie Finn investigation covers a 13-year period of DHS and contractor interaction with the Finn household. Natalie Finn died in 2016 at 66 pounds. Ten abuse reports were filed in the 11 months preceding her death. The Ombudsman found an 80% rejection rate for those reports.

The investigation is formally titled "A Tragedy of Errors" — language chosen by the Ombudsman, not by this petitioner.

12.3.2 Intake Record Falsification — "Starving" Deliberately Omitted

"Starving"... none of those statements documented in intake." (p.8)

This is among the most structurally significant findings in this section. Mandatory reporters — individuals who contacted DHS about Natalie Finn — used the word "starving" in their reports. The Ombudsman's investigation found that this specific language was not documented in the intake record. The word "starving" was stated by reporters and not recorded.

This is not a documentation gap — it is documented omission. The intake record did not fail to capture the reporter's language through error; the Ombudsman's investigation established that the statement was made and was not recorded. In a case where a child died of starvation, the word "starving" was omitted from the intake record created at the point of first contact.

12.3.3 Safety Assessment Completed Before Child Was Seen

"Safe even though she still had not seen them." (p.14)

The Ombudsman found that a safety assessment was marked "Safe" by a caseworker before the caseworker had made physical contact with the Finn children. A safety assessment is the instrument by which Iowa DHS determines whether a child is at immediate risk of harm. Completing a safety assessment — and marking it "Safe" — prior to seeing the child renders the instrument functionally meaningless as a safety tool and constitutes a documentation fraud rather than an actual assessment.

AUDIT RELEVANCE: The CFSR Round 3 PIP (Section 6.4.17.4) documented that Iowa's safety plans were routinely used as "promissory notes" rather than actual safety instruments, and that safety assessments in 91% of applicable cases were "not sufficiently in-depth." The Natalie Finn finding — a safety assessment completed before the child was seen — represents the outer limit of the assessment quality failure documented in the federal review: not an inadequate assessment, but no actual assessment at all.

12.3.4 Seventy-Eight Day Delay to First Home Visit

"78 days elapsed before... contact with Finn children." (p.16)

Following an accepted report, 78 days elapsed before a caseworker made contact with the Finn children. Iowa's CFSR Round 3 data (Section 6) documented that Item 14 (Caseworker Visits with Child) was a Strength in only 51% of cases. The 78-day delay in the Natalie Finn case represents the case-level manifestation of the systemic caseworker contact failure documented at the population level in the federal review.

12.3.5 Supervisor Admission — "I Lost Track of This Case"

"I lost track of this case." (Supervisor Beth Avery, p.101)

The Ombudsman's investigation includes sworn testimony from Supervisor Beth Avery admitting that she lost track of the Natalie Finn case. Supervisory case tracking is the primary internal accountability mechanism within DHS's case management structure. The supervisor's sworn admission that oversight of this specific case was lost — in the context of 10 abuse reports filed in 11 months — documents supervisory capacity failure at the individual case level that is consistent with the systemic pattern documented in CFSR findings and the AOS annual reports.

12.3.6 Worker Burnout Admission

"Pushing workers... may have caused noncompliance." (p.24)

The Ombudsman's investigation documents an admission that worker burnout — caused by agency workload pressure — may have contributed to noncompliance with DHS policy.

This admission, made within the context of a child fatality investigation, connects the workforce attrition and caseload increase data documented in the CFSR records (Section 6) and the AOS annual reports (Section 11) to a specific, investigated child death.

12.3.7 CSIU Intake Unit — Staffing Stasis and Structural Overload

The Natalie Finn investigation contains the most detailed operational analysis of Iowa's Central Child Abuse Intake Unit (CSIU) available in the 25-year oversight record. The findings document a unit that was structurally frozen while demand around it accelerated:

Staffing: CSIU employed 21 intake workers when it opened in 2010. By 2011 the count reached 23 — and remained at 23 for the subsequent eight years. Unit Administrator Geyer: "If everybody is here, all 23 workers... But as soon as three of those people are gone — I mean, it's staffed pretty tight, I'll say that." In the two-year period 2017–2018, at least 10 intake workers left the position. (p.23–24)

Funding requests: "Since CSIU's inception in 2010, officials have never requested an increase in funding specifically for CSIU operations." Service Area Manager Lipscomb: "With a status-quo budget, there's nothing to ask for. There's no money for additional positions." (p.23)

Caseload acceleration: Field worker assessments increased from 25,707 to 35,029 from 2016 to 2018 — a 36% increase in two years. (p.23)

Hold times: CSIU management goal had been 45 seconds average hold time. By time of investigation, average hold time was under 2 minutes — with some calls on hold for 10 to 12 minutes on busy days. (p.24)

AUDIT RELEVANCE: CSIU is the single entry point through which all Iowa child abuse reports pass. A unit staffed at 23 workers from 2011 through 2019 — with no funding increase ever requested — while field caseloads increased 36% in two years, is the structural explanation for the intake suppression pattern documented across all three Ombudsman fatality investigations. The Natalie Finn report confirms this is not an individual worker performance failure: it is an institutional resource failure operating at the system's intake gateway.

12.3.8 Training Compliance — 49% Agency-Wide; 24% for Intake Workers

Iowa law requires ongoing social work field staff to complete 24 hours of child welfare training annually. The Natalie Finn investigation documents:

"For fiscal year 2017, DHS reported that only 49 percent of 'ongoing social work field

staff' met this requirement. For the five intake workers we interviewed, their compliance with this requirement has been approximately 24 percent — substantially lower than the agency-wide figure of 49 percent." (p.24)

The 49% agency-wide training compliance rate is independently consistent with the CFSR Round 3 PIP finding (Section 6) that identified staff training as a documented area of non-conformity. The 24% rate specifically among CSIU intake workers — the personnel making the initial accept/reject decisions on every child abuse report in Iowa — documents that the workers at the system's most critical decision point had the lowest documented training compliance rate in the agency.

12.3.9 Mandatory Reporter Written Reports — Systemic Non-Review

All ten abuse reports in the 2015–2016 Finn case were made by mandatory reporters. Six of those ten mandatory reporters did not file the statutorily required written follow-up report. Of the four who did submit written reports, the Ombudsman found:

"Prior to Natalie's death, nobody at CSIU was designated to review written reports from mandatory reporters regarding rejected intakes. This means it is unlikely that anyone reviewed the statutorily required written reports submitted by the mandatory reporters in the Finn case — or any other written report from a mandatory reporter for a rejected intake before 2017." (p.25)

The statutory mechanism designed to provide a secondary documentation layer for child abuse intake — the mandatory reporter's written report — was structurally non-functional. No one was assigned to read them. CSIU implemented a new practice after Natalie's death assigning two Social Worker 4s to review these reports for rejected intakes.

12.3.10 First Independent Investigation of CSIU Since Inception

"According to DHS officials, our review was the first independent investigation of CSIU's intake process since the unit's inception in 2010." (p.25)

CSIU was established in 2010 as the centralized intake unit for all Iowa child abuse reports. The Ombudsman's 2020 investigation of the Natalie Finn case was the first independent review of that unit in its 10-year operational history. The unit that processed every child abuse intake in Iowa for a decade — including all three rejected intakes that preceded Natalie Finn's death — had never been independently reviewed until a child died.

12.3.11 Ombudsman's Concluding Warning

"We believe another tragedy could happen if DHS field and intake workers are not given

the resources and support they need and deserve." (p.25)

The 14 recommendations issued by the Ombudsman were accepted in part by DHS Director Kelly Garcia: DHS accepted 11 of 14 recommendations. Three recommendations were not accepted. The Ombudsman stated it would monitor and pursue implementation of those not accepted.

The DHS Director's response: "This was a tragic case. The Finn children should never have had to endure the treatment they received. We will learn from this and improve the safety net DHS provides to Iowa's children."

AUDIT RELEVANCE: The Sabrina Ray investigation was published seven months after the Natalie Finn investigation — September 2020 versus February 2020. Ray died in 2017, one year after Finn. The Ombudsman's warning that "another tragedy could happen" was issued in February 2020 while simultaneously preparing the Sabrina Ray investigation. The warning was accurate.

12.3.12 Explicit Continuity With Shelby Duis — Ombudsman's Own Finding

"Problems... in 2000 Shelby Duis report repeated in Natalie's case." (p.1)

This is the most structurally significant single sentence in the Natalie Finn report for petition purposes. The Ombudsman — an independent constitutional officer — explicitly states in the opening of the investigation report that the problems identified in the 2000 Shelby Duis investigation were repeated 16 years later in Natalie Finn's case. This is not an interpretive claim by this petitioner. It is the Ombudsman's own written finding.

The corrective actions recommended following the Shelby Duis investigation in 2000 did not prevent the same failure pattern from producing another child fatality investigation 16 years later.

12.4 ATTACHMENT A29 — OMBUDSMAN CASE STUDY: SABRINA RAY

Official Investigation Title: "Misplaced Trust: An Investigation of the Death of Sabrina Ray"

Source: Iowa Office of Ombudsman — Kristie Hirschman

Date: September 8, 2020

Authority: Iowa Code § 2C.15

Period Under Review: 2010–2017

Evidentiary Classification: Primary | Domains: B; C; E; F; G; H

deficiency categories: F1; F2; F6; F9; F10; F12

12.4.1 Summary of Findings — Scene and Death

On May 12, 2017, at 1708 1st Street in Perry, Iowa, Officer Josh Sienkiewicz of the Perry Police Department was dispatched to a report of a minor female not breathing. Upon arrival, he described what he saw as:

"The most horrific scene in my entire career and life." (Officer Sienkiewicz, p.3)

Sabrina Ray was found on a small mattress on the floor wearing a diaper and tank-top. Officer Sienkiewicz observed that Sabrina's body was "nearly withered out of existence." Emergency personnel considered resuscitation efforts futile. EMS noted skeletal features and extreme malnourishment. The medical examiner determined cause of death as severe malnutrition due to denial of critical care.

Sabrina Ray was 16 years old. She was 4 feet, 7½ inches tall. She weighed 56 pounds. The autopsy revealed that, based on the size and condition of her internal organs, she was likely subject to starvation over a long period of time.

The bedroom where she died had a window covered with a cover, a baby monitor-like camera, a magnetic alarm on the door, and evidence of a padlock. Screws had been placed to prevent the windows from opening. Locks and alarms had been installed on food and beverage pantries in the kitchen and family room. On the day she died, her adoptive parents were en route to Disney World.

12.4.2 Criminal Outcomes — Five Convictions

The criminal investigation resulted in charges and convictions against five family members:

- Misty Ray (adoptive mother): sentenced to life plus 20 years
- Marc Ray (adoptive father): sentenced to 80 years
- Carla Bousman (adoptive grandmother): sentenced to 20 years
- Josie Bousman (adoptive cousin): sentenced to 14 years
- Justin Ray (adoptive brother): sentenced to 10 years

12.4.3 Scale of DHS Involvement — "Plenty of Official Eyes and Ears"

The Ray household had one of the most extensive DHS contact histories of any family in the Ombudsman's investigated record:

- Licensed daycare ("Rays of Sunshine") for 13 years — 112 children cycled through
- Licensed foster care parents for nearly 10 years — 23 total foster placements accepted
- 4 children adopted from foster care, including Sabrina
- 11 child abuse reports filed across a 5-year span — 100% rejected

The Ombudsman's summary:

"DHS was already intimately familiar with the Ray family before Sabrina's death. The agency was involved with them through licensing the home as a child-development home. The Rays also were previously an approved and licensed foster care home. To the extent possible, there were plenty of official eyes and ears on this family." (p.24)

The 100% rejection rate for 11 abuse reports was not achieved by keeping DHS workers away from the household. It was achieved while DHS workers, FSRP contractors, daycare licensing staff, and foster care licensing staff were all actively engaged with the family.

12.4.4 Coached Children — Systemic Manipulation Documented

"Testimony from the surviving children confirmed what several reporters suspected — that the Rays had coached the children in their home to provide acceptable answers to DHS's questions." (p.24)

DHS did not receive honest answers during its investigations at the home. The Ombudsman documents this not as an excuse for DHS failures, but as part of the analysis of why communication among DHS units was structurally critical — workers interacting with coached children in isolation, without cross-referencing other units' concerns, could not identify the pattern that converging information would have revealed.

12.4.5 The Messersmith Whistleblower Finding — Contractor Suppression of Mandatory Reporting

FSRP services worker Shelby Messersmith worked for Mid-Iowa Family Therapy Clinic, a DHS contractor providing Family Safety, Risk, and Permanency (FSRP) services to children placed in foster care with the Rays. Beginning in September 2013, Messersmith made multiple reports to DHS expressing grave concerns about conditions in the Ray household. In June 2014, she was called into a meeting with Mid-Iowa supervisor Mark Chappelle and DHS ongoing-services supervisor Holly Smalley.

The Ombudsman's finding:

"It would be disappointing and unfortunate if Chappelle, Hoffman, and Smalley had merely disregarded Messersmith's warnings. But it is appalling and shameful that Chappelle and Smalley were actively stifling her efforts and reprimanding her for speaking up." (p.88)

Messersmith told the Ombudsman she was "completely blindsided" by the meeting. The message she received was: "Do your visits and shut up." She left her job with Mid-Iowa in July 2014, shortly after the meeting, stating she felt "defeated."

Mid-Iowa's policy required FSRP workers to "confer" with supervisors before making mandatory abuse reports. Iowa Code § 232.70 establishes mandatory reporting as a legal obligation. The Ombudsman issued a formal recommendation:

"The Ombudsman recommends that Mid-Iowa: Review its practice and policy that requires consultation between FSRP services workers and supervisors to ensure non-interference with reporting requirements, consistent with Iowa Code section 232.70." (Recommendation 1)

This is a formal Ombudsman recommendation directing a DHS contractor to stop actively interfering with a legal reporting obligation. The underlying conduct — a contractor reprimanding a worker for attempting to fulfill a statutory duty — occurred within a contract monitoring environment the AOS documented (Section 11.2) had zero performance evaluations and zero corrective action plans across the preceding 8-year period.

12.4.6 \$640,000 in Subsidies Paid During Abuse Period

"Angry that the state paid Sabrina's adoptive parents more than \$640,000 between 2006 and 2017 to care for children, while they abused some of those children." (Ombudsman Hirschman, From the Ombudsman letter)

The Ray household received \$640,000 in foster care and adoption subsidies across an 11-year period concurrent with documented abuse. DHS workers and FSRP contractors were regularly present. 11 abuse reports were filed and rejected. The total state liability arising from the Ray household was documented at \$14.2 million (\$10M settlement reported by KCCI News — A36; coverage in People Magazine — A35).

12.4.7 Foster License on "Hold" — Children Remained; No Policy Framework

The foster care license was placed on "hold" on April 11, 2014 — meaning DHS determined the home was unsuitable for new placements. Children already in the home, including foster children, remained. The Ombudsman:

"If DHS does not trust the foster home for future placements, there should be no question whether the foster home should be trusted for current placements. Keeping the children in the home should never have been an option." (p.89)

The investigation documents there was no formal policy for placing a foster family on "hold" — no articulated protocol for when children in the home should be removed, no timeline requirement, no decision framework. The absence of policy governing this specific situation — a home judged unsuitable for new placements while existing placements remain — is a structural gap that enabled the children to remain in the Ray household for three more years.

12.4.8 No Internal DHS Review Following Sabrina's Death

"We are also concerned about the lack of internal investigation by DHS following Sabrina's death. DHS provided no explanation as to why a formal review had not been completed." (p.23)

DHS conducted no internal review of its own actions and decisions following Sabrina's death. Former DHS supervisor Chappelle, when interviewed by the Ombudsman, stated he was "surprised no internal investigation took place" and that "usually after something like that, we would want to talk to everybody who touched this in the last five years."

The Ombudsman's conclusion: "Had the Ombudsman not initiated an investigation of Sabrina's death, DHS's shortcomings and failures would have gone undiscovered to both the agency and the public." (p.24)

The Child Fatality Review Committee — a body authorized by Iowa law following the Shelby Duis death in 2000 — had never been convened since its authorization. In three consecutive child fatality investigations across a 20-year arc, the only entity that conducted an independent review of DHS's actions was the Ombudsman's office.

12.4.9 Thirteen Recommendations — DHS Accepted 10 of 13

The Sabrina Ray investigation issued 13 formal recommendations. DHS accepted 10 of 13. The three recommendations not accepted included: amending the records retention schedule for child abuse intakes, and removing foster children as soon as possible when a home is placed on hold.

12.4.10 The Third Case — Disclosed But Never Published

The Ombudsman's letter discloses: "Sadly, there is a third case of a child death that we are continuing to investigate." (p.1) At the time of publication (September 2020), a third DHS-involved child fatality investigation was underway and publicly disclosed.

The full investigative archive of the Iowa Office of Citizens' Aide/Ombudsman, as maintained on the official legislative portal, contains no published report for a third child fatality investigation following Natalie Finn and Sabrina Ray. As of May 2026, six years after the Sabrina Ray report's disclosure of an ongoing third case, no corresponding report has been published.

Based on publicly available contemporaneous reporting, the case most consistent with the described timeframe is that of Malayia Knapp — a 17-year-old who fled her adoptive home in Urbandale, Iowa on December 1, 2015, after suffering years of documented physical

abuse, starvation, and confinement by her adoptive parents, Mindy and Anthony Knapp.

Malayia survived. Her adoptive mother was subsequently convicted of child abuse.

Following Malayia's escape, DHS removed two younger siblings from the same home.

Malayia filed a civil lawsuit against Iowa DHS in 2017 seeking \$500,000 in damages.

The Knapp case falls within the seven-month window described by the Ombudsman — Malayia fled December 2015, Natalie Finn died October 2016, and Sabrina Ray died May 2017.

All three cases involved: adopted children, DHS prior contact history, abuse occurring after permanency was established, and homeschooled or otherwise isolated children shielded from mandatory reporter contact.

That the Ombudsman described this as a "child death" investigation while Malayia survived may indicate the Ombudsman was simultaneously investigating a separate death case in the same cluster period not yet identified in public records — or that the case description in the letter was imprecise. This petition notes the discrepancy and the absence of a published report, and identifies the unresolved status of the third investigation as an independent basis for audit inquiry.

AUDIT RELEVANCE: The Iowa Auditor of State is authorized under Iowa Code § 11.6 to audit the practices and records of state agencies. The non-publication of a disclosed, publicly announced Ombudsman child fatality investigation — six years after its disclosure — is itself an auditable event. The Auditor may inquire: Was the investigation completed? If so, why was no report published? If not, what occurred that precluded completion? What DHS records were accessed, and what findings, if any, were shared with DHS leadership without public disclosure?

12.4.11 The Post-Permanency Blind Spot — Sabrina Ray as Direct Evidence

Sabrina Ray was adopted from the Ray foster home — a permanency outcome. Her adoptive placement was with the same household that had been licensed by DHS as a foster home for nearly 10 years and had received \$640,000 in public subsidies. The monitoring infrastructure that existed during the foster care period — FSRP services, daycare licensing inspections, ongoing-services workers, field workers — was the same infrastructure that failed to protect her after permanency was achieved.

The post-permanency blind spot, as documented in this petition (Section 7 and Section 14), is the structural failure of oversight mechanisms once a permanency outcome has been established. The Sabrina Ray case demonstrates this failure at its most acute: permanency was achieved through adoption into the abusive household, and the post-permanency oversight infrastructure — despite continuous agency presence — did not detect or act on

conditions that ultimately killed her.

12.5 LONGITUDINAL SYNTHESIS — THE TRINITY PATTERN AND ITS

SYSTEMIC IMPLICATIONS

12.5.1 The 25-Year Failure Arc

The three fatality investigations in this section, together with the 15-year annual report series, document a continuous 25-year oversight failure arc:

2000 — Shelby Duis: 3 rejected reports; hotline records destroyed; non-intake staff screening calls; supervisors with no staffing to review evidence.

2015 — Malayia Knapp (Urbandale): Survived after fleeing adoptive home December 1, 2015. Physical abuse, starvation, and confinement by adoptive parents Mindy and Anthony Knapp. DHS prior contact history. Adoptive mother convicted. Two siblings later removed from same household. Civil lawsuit filed against Iowa DHS, 2017. Third Ombudsman fatality investigation disclosed September 2020; no report published as of May 2026.

2016 — Natalie Finn (West Des Moines): 10 reports / 80% rejection; "starving" omitted from intake; safety assessment completed before child seen; 78-day delay; supervisor lost track of case.

2017 — Sabrina Ray: 11 reports / 100% rejection; contractor actively suppressed mandatory reporting; DHS workers regularly present with suspicions documented; \$640K paid during abuse period; foster license on hold while children remained.

2025 — Annual Report: 6,266 cases / 57% increase since 2014; due process violations; geographic inconsistency; communication restrictions without evidentiary support.

12.5.2 The Ombudsman's Own Continuity Statement

The structural anchor for this section is the Ombudsman's own statement in the Natalie Finn investigation: "Problems... in 2000 Shelby Duis report repeated in Natalie's case."

(p.1) This statement was made by the independent constitutional officer who conducted both investigations — not by this petitioner, not by an advocacy organization, and not by an interested party. It is an official government finding.

Between the Shelby Duis recommendations in 2000 and the Natalie Finn death in 2016, Iowa completed two full CFSR cycles with Performance Improvement Plans. Each PIP addressed intake practices, supervisor capacity, and caseworker contact requirements. None of the federal corrective action cycles prevented the recurrence of the same pattern the Ombudsman identified in 2000.

The Natalie Finn investigation issued 14 formal recommendations. The Ombudsman confirmed this was the first independent review of CSIU in its 10-year history. DHS accepted 11 of 14 recommendations. Seven months later, the Ombudsman published the Sabrina Ray investigation. The corrective actions initiated after the 2000 Shelby Duis recommendations, after three CFSR cycles, and after the post-Finn DHS reforms all failed to prevent the conditions documented in the Sabrina Ray investigation from existing simultaneously.

12.5.3 The Contractor Accountability Connection

The Sabrina Ray finding — contractor suppression of mandatory reporting — is not an isolated contractor misconduct finding. It is documentably connected to the AOS Service Contract Monitoring Report (Section 11.2), which documented that across 2010–2018, DHS had zero performance evaluations, zero deficiency communication, and zero corrective action plans for contracted service providers. A contractor that required workers to "confer" before making mandatory abuse reports operated within a monitoring environment the Auditor of State documented had no accountability infrastructure. These two findings are causally linked, not merely temporally proximate.

12.5.4 The Intake Suppression Pattern as Systemic Finding

Across all three fatality investigations, the Ombudsman identified intake suppression as a primary mechanism: abuse reports were rejected, language was omitted from records, and assessments were completed before children were seen. In the CFSR Round 3 review (Section 6), federal reviewers found that Item 1 (Timeliness of Initiating Investigations) was rated a Strength in only 71% of cases, and the PIP root cause analysis documented "inconsistent application of safety vs risk concerns related to removal of children."

The federal review measured aggregate timeliness. The Ombudsman's fatality investigations measured intake suppression at the individual case level. Both sources document the same structural deficiency — the point at which the Iowa child welfare system first receives information about a child at risk and decides whether to act on it — from different methodological vantages, producing convergent findings.

12.6 CROSS-REFERENCES

Section 6 — CFSR Longitudinal Forensic Analysis (intake and caseworker visit data)

Section 11 — AOS Financial Oversight (contract monitoring — Sabrina Ray connection)

Section 13 — Fatality Risk Analysis (A30)

Section 14 — Case-Level Outcomes (Drew household — post-permanency pattern)

Section 13 — Systemic Failure Matrix (F1, F2, F3, F9, F12 convergence)

Attachment A26 — Ombudsman Annual Reports 2010–2025

Attachment A27 — Ombudsman: Shelby Duis Investigation

Attachment A28 — Ombudsman: Natalie Finn Investigation ("A Tragedy of Errors")

Attachment A29 — Ombudsman: Sabrina Ray Investigation ("Misplaced Trust")

Attachment A30 — Fatality Risk Analysis

Attachment A35 — People Magazine (Sabrina Ray \$10M settlement)

Attachment A36 — KCCI News (Sabrina Ray — \$14.2M total liability)

Official Investigation Title: "Investigation of the Department of Human Services' Handling of Allegations of Child Abuse Concerning Shelby Duis"

Source: Iowa Citizens' Aide/Ombudsman — William P. Angrick II

Date: December 14, 2000

Authority: Iowa Code Chapter 2C; Senate request from President Kramer, Majority Leader Iverson, and Senator Tinsman

Period Under Review: 1996–January 4, 2000

Investigation Initiated: February 10, 2000 (5 weeks after Shelby's death)

Evidentiary Classification: Primary | Domains: B; C; E; F

deficiency categories: F1; F2; F6; F12

12.2.1 Death and Background

Shelby Duis was found dead at her home in Spirit Lake, Iowa on January 4, 2000. She was 2 years and 9 months old. She lived with her mother, Heidi Watkins, Watkins' boyfriend Jesse Wendelsdorf, and her half-brother Tyler. Autopsy findings documented:

- Multiple blunt force injuries to various parts of Shelby's body
- Both acute injuries and older injuries inflicted weeks and months before death
- Fractured metacarpals (at least two separate episodes, both in December)
- Fractured ribs occurring "two to six weeks" before death

Watkins and Wendelsdorf were charged with first-degree murder and felony child endangerment. A jury found Wendelsdorf not guilty. Watkins was convicted of multiple acts of child endangerment (class B felony) and acquitted on the murder charge.

12.2.2 Pre-Existing Methamphetamine History — Known to DHS Before Shelby's Birth

The Shelby Duis case is the foundational document in this petition for the proposition that

Iowa DHS had a documented, systemic failure to integrate substance use history into child safety decision-making — a failure that began decades before Ashley Meredith's 2018 case.

June 1996: A DHS intake was accepted after a report that methamphetamine powder and rocks were observed on a kitchen table in the presence of two toddlers (ages 2 and 2 months). Watkins was babysitting those children. The investigating worker, Illg, concluded Watkins was not credible and documented: "This office has had previous concerns about [Watkins], all of which have involved illegal drug use." The report was founded for Denial of Critical Care. No services were recommended.

June 1997: Watkins was criminally charged with two counts of possession of controlled substances (methamphetamine and marijuana) and one count of child endangerment.

March 1997: Shelby Duis was born — after the 1996 founded DHS report and during the criminal proceeding.

June 1998: The charges against Watkins were dismissed after compliance with deferred prosecution terms.

AUDIT RELEVANCE: DHS had a founded abuse report for drug-related child endangerment against Shelby's mother before Shelby was born. This pre-existing substantiated record did not generate a safety protocol for the child born into that household. The absence of a longitudinal risk integration mechanism — treating prior founded reports as baseline safety context for subsequently born children — is documented here as a 2000 failure and remains unaddressed through the entire CFSR arc of 2003–2024.

12.2.3 DHS Internal Review — Institutional Self-Exoneration

Following Shelby's death, DHS initiated its own internal review on February 2, 2000. In a report released to the public on March 3, 2000, DHS identified six areas for improvement, including staffing and caseloads, training, and community education. However, DHS concluded:

"From the review of Shelby's death, we determined that the policies critical to her safety were followed." (DHS Internal Review, March 3, 2000)

This conclusion is directly contradicted by the Ombudsman's independent investigation, which identified 23 specific failures across reporting, intake, and assessment processes. The institutional self-exoneration documented in the DHS internal review is the structural predecessor of the CFSR self-assessment dynamic documented in Section 6: the state reported conformity or compliance through internal mechanisms while external review documented pervasive failures.

12.2.4 Decentralized Intake — Root Cause and Root Recommendation

The Ombudsman's 2000 report identifies the decentralized intake structure as the systemic root of Iowa's child protection failures:

"Streamlining how reporters interact with the DHS child abuse system and dedicating a centralized unit of uniformly trained intake workers would go a long way toward resolving individual differences and regional variances found in the current decentralized intake system. It could ensure consistent, accurate, and appropriate responses to the initial reports of child abuse or neglect." (Executive Summary, p.iii)

Recommendation 1 of the 2000 Ombudsman report — the single most fundamental structural recommendation — called for DHS to redesign the child abuse reporting system with a single point of contact for all reporters, a centralized intake unit, and uniform documentation standards. This recommendation was issued in December 2000.

Iowa did not implement a centralized intake unit (CSIU) until 2010 — a decade after the recommendation. The 2020 Natalie Finn investigation documents that CSIU was never independently reviewed from its inception in 2010 until after Natalie's death. The centralization that the 2000 Ombudsman recommended to prevent intake failures was itself underfunded, understaffed, and unreviewed for its entire first decade of operation.

12.2.5 Specific Intake and Assessment Failures

The Ombudsman found the following specific failures in the handling of abuse reports on Shelby Duis (summarized from Executive Summary and full findings):

- Two reports of child abuse were not handled or documented as intakes at all.
- Three reports should have been accepted for investigation but were rejected. The last rejected report occurred three weeks before Shelby died.
- In three instances involving alleged physical abuse, intake documentation did not sufficiently describe the reported injuries.
- One report should not have been rejected without first conducting an in-depth intake (contacting mandatory reporters with relevant information).
- None of the mandatory reporters who made calls concerning Shelby submitted the required written follow-up reports.
- Multiple mandatory reporters had difficulty reaching intake workers and experienced unreturned calls and non-documented contacts.
- After-hours hotline contact reports were filed by the State Training School in Eldora and then destroyed after three months — DHS's Bureau of Protective Services chief was

unaware the records existed until the Ombudsman discovered them.

12.2.6 Drug Testing Referral — Ordered, Never Implemented

On October 22, 1999, the CPW (Illg) recommended and Watkins agreed to accept in-home services and submit to random drug testing. Shelby died on January 4, 2000.

"Drug testing was never arranged or conducted." (p.118)

In-home services were not approved until December 15, 1999. The provider's first visit to Watkins' home did not occur until December 23, 1999 — 12 days before Shelby died.

"The Ombudsman is concerned that no drug testing took place, especially in light of the evidence revealed at the criminal trials that Watkins was smoking marijuana and methamphetamine." (p.118)

AUDIT RELEVANCE: The Shelby Duis case documents that in 2000, the failure mode for drug testing was: testing was agreed to and never implemented because of miscommunication between workers in the referral process. Iowa HHS Communication 626 (October 2025, A52) documents that in 2025, the failure mode is: drug testing results cannot serve as the sole basis for TPR and sweat patch results require laboratory confirmation. Twenty-five years separate these two documents. Both concern the same fundamental gap: the state's inability to properly administer drug-related evidence in child welfare cases.

12.2.7 The Origin of the Child Fatality Review Committee Authorization

Following Shelby Duis' death and the Ombudsman's 2000 investigation, Iowa lawmakers approved a new law authorizing the creation of an ad hoc Child Fatality Review Committee (CFRC) to independently review DHS-involved child fatalities. The law authorizes the State Medical Examiner to establish a CFRC to "immediately review the child abuse assessments which involve the fatality of a child under age eighteen ... to determine whether the department of human services and others involved with the case of child abuse responded appropriately."

The CFRC was authorized in direct response to Shelby Duis's death.

It has never been convened. In both the Natalie Finn investigation (February 2020) and the Sabrina Ray investigation (September 2020), the Ombudsman issued formal legislative notifications that the CFRC had never been convened and called on the Legislature to re-evaluate its expectations for child fatality oversight bodies.

As of December 2024 — the date of CFSR Technical Bulletin 14 — the CFRC authorized by the Iowa Legislature in response to the year-2000 death of a 2-year-old remains dormant.

12.2.8 Twenty-Three Recommendations — The Foundation Document

The Ombudsman issued 23 recommendations in the Shelby Duis investigation, spanning: intake system redesign, mandatory reporter obligations, documentation standards, collateral contact requirements, supervisor review requirements, drug testing protocols, medical expert access, and multi-disciplinary team utilization.

Recommendation 15 explicitly called for DHS to provide training on methamphetamine and how it impacts family dynamics and child safety — the first formal state-level recommendation recognizing meth as a child welfare risk factor. This recommendation was issued in the year Shelby Duis died. It predates by 25 years the CFSR Round 4 period in which Iowa HHS still lacks adequate SUD treatment coordination infrastructure (Section 9).

The 23 Shelby Duis recommendations are the documentary baseline against which all subsequent CFSR PIPs, Ombudsman investigations, and external evaluations in this petition must be read. Every subsequent failure documented in this section (Natalie Finn: 14 recommendations; Sabrina Ray: 13 recommendations) is a failure to hold against the 2000 baseline — not a fresh, isolated system deficiency.

12.X DOMAIN & DEFICIENCY MAPPING — SECTION 12

Domain D — Judicial Interface & Legal Oversight:

F11 — Interagency Coordination Failure: Ombudsman fatality investigations in Duis (2000), Finn (2017), and Ray (2018) each document multi-agency coordination failures as primary contributing factors — across a span of 18 years with no documented structural correction to the coordination architecture.

F12 — Supervisory Review & Escalation Failure: All three fatality investigations document supervisory review failures as proximate causes — workers acted without adequate oversight; supervisors failed to escalate known risk indicators.

Domain C — Decision Controls & Case Authorization:

F1 — Case Planning & Service Delivery Failure: Duis, Finn, and Ray each document failures in service planning that preceded the fatal outcomes — inadequate safety plans, missed service connections, and insufficient case review frequency.

F3 — Assessment & Decision-Making Deficiency: All three cases involve

documented risk indicators that were present in the record but not actioned — consistent with the "promissory note" safety plan finding in the Round 3 PIP.

Domain G — Federal Oversight History & Corrective Action Response:

F5 — Corrective Action Failure: The 23 Shelby Duis recommendations (2000) establish the documentary baseline. Natalie Finn (14 recommendations, 2017) and Sabrina Ray (13 recommendations, 2018) represent documented recurrence of substantially the same systemic failures 17–18 years later. No CFSR PIP between 2000 and 2018 produced sustained correction.

Domain B — Contract & Provider Oversight:

F2 — Ongoing Monitoring Deficiency: Finn and Ray involve licensed providers or supervised placements. Monitoring failures are documented as structural contributors in both investigations.

12.X AUDIT QUESTIONS GENERATED — SECTION 12

AUDIT QUESTION 12-A: The Shelby Duis investigation (2000) produced 23 formal recommendations. What is the documented implementation status of each recommendation, and what Iowa DHHS records exist showing the disposition of each recommendation after issuance?

AUDIT QUESTION 12-B: Natalie Finn (2017) and Sabrina Ray (2018) each produced fatality investigation reports with systemic findings substantially overlapping the Duis recommendations from 2000. What internal root cause analysis, if any, did Iowa DHHS conduct to explain why the intervening 17 years of CFSR oversight, PIP compliance activity, and Ombudsman reporting did not prevent the recurrence of substantially identical systemic failures?

AUDIT QUESTION 12-C: What is Iowa DHHS's current protocol for tracking the implementation status of Ombudsman fatality investigation recommendations, and what entity bears responsibility for verifying implementation — the Ombudsman, Iowa DHHS, or a third party?

AI assistance disclosure: See Section 5.8.

reports identified in the attachment citations above.

End of Section 12 — Version 1.1 — June 5, 2026

Audit Petition Packet v2.0 (5/2026)

AI assistance disclosure: See Section 5.8.

AI assistance was used materially in the preparation of this document. That assistance does not constitute independent verification of any factual claim.